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Journal of Affective Disorders 88 (2005) 209–215

JOURNAL OF AFFECTIVE DISORDERS

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Brief report

Anxiety and depression in the workplace: Effects on the individual and organisation (a focus group investigation)

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Received 4 January 2005; received in revised form 22 July 2005; accepted 22 July 2005

Abstract

Background: While the prevalence of anxiety and depression has increased, little is known of the impact on working life. The aim of this study was to explore the effects of anxiety and depression and the treatment for these conditions on performance and safety in the workplace.
Method: Nine focus groups were conducted with employees who had suffered anxiety and depression. A further 3 groups comprised staff from human resources and occupational health. The sample comprised 74 individuals aged 18–60 years, from a range of occupations. Results were presented to a panel of experts to consider the clinical implications.
Results: Workers reported that the symptoms and medication impaired work performance, describing accidents which they attributed to their condition/medication. Respondents were largely unprepared for the fact that the medication might make them feel worse initially. Employees were reluctant to disclose their condition to colleagues due to the stigma attached to mental illness.
Limitations: People who had experienced problems with managing their symptoms and medication at work are more likely to volunteer to participate in such a study than those who had a satisfactory experience. Also, the researchers had no background information on severity of mental health problems of participants.
Conclusions: Anxiety and depression were associated with impaired work performance and safety. The authors consider the implications for health care and the management of mental health problems at work.

Keywords: Anxiety; Depression; Medication; Work; Safety

1. Introduction

Anxiety and depression has increased sharply in recent years (HSC, 2004), and prescriptions for medication have also increased. Laboratory studies show

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doi:10.1016/j.jad.2005.07.009

that psychotropic medicines impair attention, memory and motor coordination (Potter, 1990), but it is unclear how this translates to the workplace (Tilson, 1990). Lack of treatment may also be a problem as employees suffering with anxiety or depression are likely to experience symptoms (e.g. fatigue and poor concentration) that impair performance. This research used a qualitative approach to collect data on anxiety and depression and the use of psychotropic medication among the working population.

2. Method

Twelve focus groups were used to collect information on the experiences of people with anxiety and depression. Various recruitment techniques were used: liaison with established contacts in organisations and trades unions; mail shots, telephone calls and emails to organisations; advertisements in newspapers, professional publications and local radio; and distribution of posters to organisations. Established organisational contacts, advertisements and posters produced most respondents.

Twelve focus groups were conducted. Nine involved people with personal experience of anxiety/depression in the previous 2 years and who had taken prescribed medication during that period. No information was gathered on the presence of physical illnesses or other types of medication. The groups comprised:

- Individuals from health care, social services, education, manufacturing, engineering, retail, service industries (6 groups),
- Individuals attending anxiety management courses (3 groups).

Three focus groups were conducted with staff having responsibility for human resources, occupational health and health and safety. The aim was to examine organisational policy relating to mental health. The results from the 12 groups were presented to a panel of experts to consider the clinical implications.

Two focus group interview schedules were developed, one for employees and one for organisational representatives. Each discussion lasted for approximately 90 min and was recorded. The recorded material was transcribed and analysed by sorting verbatim material into themes using the method described by Knodel (1993). Reliability was ensured through systematic review of the data by 3 researchers.

3. Results

Demographic profiles of participants ($n = 74$) are shown in Tables 1 and 2. Table 3 lists areas of expertise of the expert panel. Table 4 shows the medication taken and proportion receiving each class of drug.

The physical symptoms included: nausea, headaches, dizziness, trembling, insomnia and lack of energy. Psychological symptoms involved: poor concentration, extreme emotional distress and lack of motivation. Respondents reported that they were unable to concentrate or to make decisions. Participants found it difficult to distinguish between the symptoms and the medication as both were reported to lead to: confusion, dizziness, shaking, nausea, sleep disturbance and difficulties with decision making. Work performance was thought to suffer due to the symptoms and medication. A 43-year-old female consultant psychiatrist explained:

"I get very indecisive, which isn't very good when you are meant to make decisions and then I can feel myself getting pushed into making hasty and perhaps wrong decisions."

Non-compliance with medication was common (over 2/3rds of the sample) due to unpleasant side effects, lack of improvement in symptoms or because the medication initially made users feel worse. Respondents were largely ill prepared for their medication and would have welcomed more information from doctors. Some people took less than the prescribed amount or stopped taking the medication due to concerns about dependency.

Workers described a range of accidents, which they attributed to their condition or medication, including industrial injuries and falls:

"I had a series of falls at the time... I would go faster and faster and faster. If I'd got a large queue, I'd try and deal with them really quickly and then, because I know they're all waiting I wouldn't take a break. I'd go faster and faster and I'm sure I would walk faster and faster. I had a series of falls and they actually sent me for some tests because of it.” [female working in university administration]

The risks associated with self-medication, such as alcohol, herbal products and caffeine, were discussed by the expert panel. Alcohol was identified as a particular work hazard, self-medication with herbal products may involve larger doses than recommended and excessive use of caffeine was also considered a problem.

Respondents felt stigmatised because people did not understand conditions like anxiety and depression. Recognition of anxiety and depression as genuine illnesses was thought to be a first step in dealing with mental health problems in the workplace. Respondents commented that ‘anxiety and depression should be as easy to talk about as a cold’. Participants felt that there needs to be increased awareness and understanding from managers.

Respondents noted that lack of flexibility meant there was little scope for maintaining people with anxiety and depression at work. Organisational representatives commented that the extent of support offered varied according to the size and culture of the organisation. Some employees stated that they had very supportive managers who offered work modification, reduced hours, redeployment, etc., to enable sufferers to keep working. But many (over 75%) iden-

Table 1
Employee focus group participant details

Group	Gender	Employment	Age range mean (S.D.)
Anxiety management group 1	8 female	Secretarial, Sales, Unemployed (2) Photographic assistant, Care assistant, Sales consultant, Prevocational tutor	18–55 36 (14.3)
Anxiety management group 2	5 female 3 male	Scientific officer (NHS), Retired (2), Administrator, Veterinary Surgeon, Car mechanic, Electrician	28–63 47 (13.3)
Anxiety management group 3	3 male 1 female	Programme Area Manager/Lecturer, Primary School Head Teacher, Unemployed, Researcher	22–55 40 (15.5)
Employee Group 1	2 female 5 male	Teacher (3), University administrator, Teacher in higher education, Registrar in primary school, University technician	35–60 45 (9.3)
Employee Group 2	7 female	Admin in university (2), Bank, Council worker, University administrator, Enquiry officer—police station, Public sector	29–53 44 (8.2)
Employee Group 3	5 female 2 male	Car leasing—payroll, Food manufacturing, Advisory teacher, Accounts—coach company, Personnel, Mental Health Social Worker, Freelance Lecturer	30–56 45 (10.2)
Employee Group 4 [Managers]	3 female 2 male	Front line manager—food company, Personnel manager (NHS), Health and safety manager (2), Head teacher, Worker in higher education	33–58 48 (8.9)
Employee Group 5 [Managers]	2 female 2 male	Lawyer in Civil service, Veterinary Surgeon, Deputy governor of prison, Computing services at university	45–54 48 (3.8)
Employee Group 6	3 female 1 male	Locum staff grade psychiatrist, Consultant psychiatrist, Former GP, Senior House Officer	28–54 40 (11.16)

tified management methods as contributory factors in their anxiety and depression and felt that unmanageable workloads had contributed to the development of their mental health problems. When managers were approached, they were often dismissive because no one else had indicated that they were having problems. Colleagues were identified as providers of practical support and an opportunity for sufferers to talk about their problems. Support networks were often lost when people were absent through sickness.

Organisational representatives mentioned various methods of rehabilitation, including: phased returns to work, therapeutic hours over 4 to 6 weeks starting with as little as 2 h a day or excusing staff from 'frontline' duties such as customer contact. Participants felt that occupational health staff should identify the implications for work of any medication taken. Liaison between employers and physicians about rehabilitation was mentioned as enabling workers to return to work earlier. The expert panel felt that prolonged absence from work removes the person's social network and can exacerbate problems. Organisational representatives noted that employees returning to work on reduced hours should be 'extra' to the staff needed, to avoid other workers being put under pressure through having to cover the shortfall. They felt that organisations need to appreciate that such measures will ultimately result in improved staff morale and productivity along with reductions in sickness absence rates and staff turnover.

Table 3
Range of disciplines covered by the expert panel

Validation group panel of experts
Union representative—MSF (Manufacturing, Science and Finance)
Psychiatrist and academic researcher
Health Safety and Environment Advisor
Occupational Physician
Professor of Occupational Health Psychology
General Practitioner
Senior lecturer and researcher in mental health in the workplace
Union representative—UNISON (Public service union)
Clinical Psychologist who runs anxiety and depression management courses

4. Discussion

The physical and psychological symptoms of anxiety and depression were reported to impair work performance and increase the risk of accidents. Non-

Table 2
Organisational representatives

Group	Gender	Employment
Organisational group 1	Male	Occupational Health Nurse—manufacturing
	Male	Occupational Health Physician—manufacturing
	Female	Occupational Health and Safety Advisor—manufacturing and construction
Organisational group 2	Male	Training Manager—Human resources development
	Male	Hospital Human Resources Manager
	Female	Counselling and Support Unit Manager
	Female	Human Resources Manager in SME
	Female	Senior Human Resources Advisor—healthcare
	Female	Personnel Officer—education
	Female	Senior Personnel Officer–education
Organisational group 3	Male	Occupational Health Nurse—manufacturing
	Male	Occupational Health Nurse—manufacturing
	Male	Union Health and Safety Representative
	Female	Occupational Health Nurse—manufacturing
	Female	Health and Safety Officer—borough council
	Female	Occupational Health Nurse—health care
	Female	Occupational Health Nurse—ambulance service
	Female	Nurse Specialist (Occupational Health)—healthcare
	Female	Occupational Health Nurse—health care
	Female	Occupational Health Nurse Specialist—civilian and service employees

Table 4
Summary of psychotropic medication

Drug group	% respondents prescribed this class of drug	Drug	Proprietary name
Selective serotonin reuptake inhibitors	72	Citalopram	Cipramil
		Fluoxetine	Prozac
		Fluvoxamine Maleate	Faverin
		Paroxetine	Seroxat
		Sertraline	Lustral
Benzodiazepines	30	Diazepam	Valium
		Lorazepam	Ativan
		Temazepam	
Anti depressants	19	Venlaxafine	Efexor
		Mirtazapine	Zispin
		Nefazodone Hydrochloride	Dutonin
		Flupentixol	Fluanxol
Tricyclics	9	Imipramine Hydrochloride	Tofranil
		Amitriptyline Hydrochloride	Lentizol/Triptafen/Triptafen-M
		Dothiepin Hydrochloride	Prothiaden
		Doxepin	Sinequan
Anti manic	6	Lithium Citrate	Li-Liquid/Priadel
		Lithium Carbonate	Camcolit/Liskonum/Priadel
Beta blockers	4	Oxprenolol Hydrochloride	Slow-Trasicor (Modified release)
			Trasidrex (with diuretic)
		Pindolol	Visken
			Viskaldix (with diuretic)

NB: some respondents were prescribed more than one class of drug.

compliance with medication was common, which may lead to poor control of symptoms and impaired work performance. The side effects of medication were considered to be similar to the symptoms of anxiety and depression. This finding concurs with a growing body of evidence which suggests that even newer generation antidepressants are by no means free of side effects (Glenmullen, 2000). People in this study were unprepared for the fact that their medication might initially make them feel worse.

Most respondents believed that unmanageable workloads contributed to their anxiety and depression. While some had supportive managers, many workers felt that their managers offered little help. Respondents felt stigmatised and were reluctant to tell people at work about their illness. They perceived a lack of understanding about the nature of anxiety and depression among their colleagues and managers. This study concurs with research showing that people hold negative beliefs about depression (Jorm et al., 1997) and that managers are more likely to believe sick-notes declaring a physical rather than mental illness (Manning and White, 1995).

The results suggest that anxiety and depression and medication impact on work. At the individual employee level this leads to impaired work performance, accidents and sickness absence. At the organisational level there are likely to be effects on productivity, staff morale, accidents, absences and staff turnover. Fig. 1 represents these inter-relationships as a schematic model. Based on the results, the authors outline some recommendations for workplace and health care practices.

Workplace practices:

- mental health issues should be an integral part of health and safety training,
- organisations should conduct risk assessments relating to mental health,
- maintaining workers with anxiety and depression at work or rehabilitating workers following sickness absence requires coordination between managers

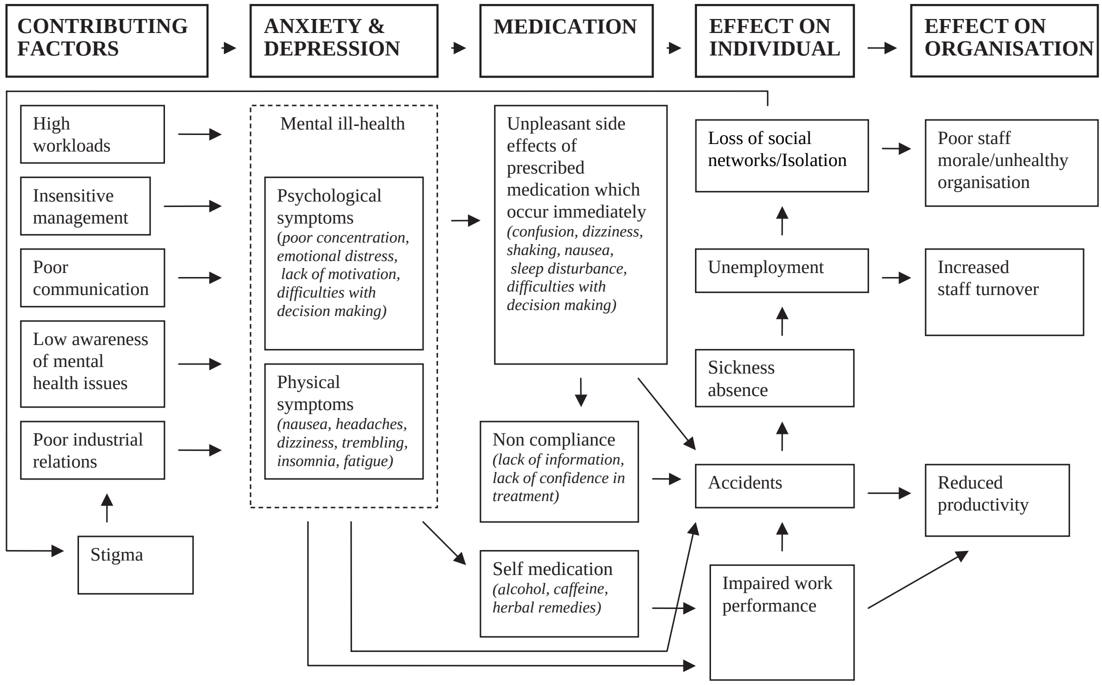

Fig. 1. Schematic model of the factors contributing to anxiety and depression and the impact of these conditions on work.

and staff from occupational health and health and safety.

Health care practices:

- there is a need for careful patient monitoring during treatment,
- patients need accessible information leaflets, explaining the actions of medication and potential impact on safety,
- more liaison is required between health care professionals and employers,
- employees should be actively involved in treatment and rehabilitation.

This study relied on volunteers and it is reasonable to assume that employees who had problems managing their symptoms/medication would be more likely to volunteer. Also, as respondents were recruited via workplaces and anxiety and depression management courses, the researchers had no information on severity of mental health problems. However, this study provides some insight into the problems faced by employees with anxiety and depression.

Acknowledgements

This work was funded by the UK Health and Safety Executive. The views expressed are those of the authors and do not necessarily reflect HSE policy. We would like to thank our focus group participants for their support and for the time they gave so generously to this study.

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